

Surg Path Final Report

* Final Report *

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TCGA-FY-A3WA-01A-PR

Redacted



* Final Report *

SURG PATH FINAL REPORT

SURGICAL PATHOLOGY

Collected:

Accession:

Physician:

PROCEDURE

Right thyroid lobectomy.

SPECIMEN:

Right thyroid node. NO FS. Fresh specimen.

HISTORY

Right thyroid nodule.

GROSS

Received fresh in a container labeled "right thyroid nodule" is a portion of thyroid (4.5 x 3 x 1.8 cm, 10.3 g). The parenchyma has a 2.1 cm in greatest dimension discrete nodule with a uniform hemorrhagic cut surface. A portion of the nodule and adjacent inked capsule is submitted for the ICDO-3
carcinoma, papillary, follicular variant 8340/3
Site: thyroid, NOS c73.9
51472 RD
The remaining firm, red-brown thyroid parenchyma has additional small nodules up to 0.4 cm in diameter. The margins of excision are marked with black ink. The tissue is submitted in its entirety in six cassettes.

MICROSCOPIC

See diagnosis.

DIAGNOSIS

Right thyroid lobe, right thyroid lobectomy (10.3 g):
Tiny incidental nodule (0.5 cm) worrisome for papillary thyroid microcarcinoma (see comment).
Nodular hyperplasia (multinodular goiter).
One parathyroid gland.

COMMENT

A tiny 0.5 cm focus is composed of follicles lined by cells that appear slightly enlarged, somewhat irregular, with pale chromatin, small nucleoli, occasional nuclear grooves and a rare intranuclear inclusion. There is no significant nuclear crowding. There is prominent associated fibrosis and the cytologic features are worrisome for a diagnosis of follicular variant of papillary thyroid microcarcinoma. A CK19 immunohistochemical stain is focally positive within this focus, however focal CK19 positivity is also

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Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
IPAA Discrepancy		
Prior Malignancy History		
Local/Synchronous Primary		
Case is (circle):		
reviewer initials	QUALIFIED	DISQUALIFIED
Date Reviewed:	10/31/12	

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identified throughout the rest of the section and is of doubtful significance. This case will be sent out to the
for consultation.

Electronic Signature)

Completed Action List:

* Order by

Type:

Date:

Status:

Title:

Encounter info:

Auth (verified)

SURG PATH FINAL REPORT

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1st Addendum Report

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SP 1ST ADDENDUM REPORT

SURGICAL PATHOLOGY

Collected:
Accession:
Physician:

ADDENDUM REPORT DIAGNOSIS:

Thyroid gland, right lobe, resection: Nodular hyperplasia.
Incidental papillary thyroid microcarcinoma (0.5 cm), follicular
variant, confined to the thyroid and completely excised.
Normocellular parathyroid gland.

*99% follicular variant
per TSS /a*

COMMENT:

I have reviewed the slides from the above named -year-old who
underwent right thyroid lobectomy for a nodule. As indicated, I
entirely agree with your diagnosis. Sections from the right thyroid
lobectomy demonstrate multiple hyperplastic nodules. In addition,
there is a 0.5 cm nodule with associated stromal sclerosis
demonstrating nuclear changes of papillary thyroid carcinoma with
crowding, chromatin clearing, and nuclear contour irregularities.
This lesion is confined to thyroid and appears narrowly excised.
Finally, there is a cellular-appearing portion of parathyroid gland.

(Electronic Signature)

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